

HEALTHBRIDGE · UK PRIMARY CARE

# HealthBridge

Smarter care, better service —
starting with the letter.

19p1362.github.io/portfolio/healthbridge

Gonela Abhilash · CPC · registered pharmacist · MSc Public Health, ARU

Independent project · concept deck · no patient data shown

ABOUT US

# Built for the work between the appointments

## Who we are

HealthBridge is built by Gonela Abhilash — a CPC-certified risk-adjustment coder, registered pharmacist and MSc Public Health student at Anglia Ruskin. His day job is turning clinical records into data a system can use.

## The problem we start from

A hospital writes one letter. A practice has to read it, decode it, code it and file it — often days after a clinician needed it. The letter is where care data goes missing.

## What drives us

- Fewer handoffs — one record, not six notes
- Records the practice owns and can export
- Change you can measure in a month, not a roadmap

★ Stated plainly: no deployed NHS customer yet. What follows is what we build and what we would measure.

## Who it is for

GP practices and primary-care networks in England, community pharmacies, and small clinics moving off paper.

SERVICES

# What we do, and how it runs better

## What we do

- Read clinical correspondence — PDF or photo — and extract diagnosis, medication and action
- Propose a SNOMED CT code for each fact, with the confidence shown
- Require a clinician to accept, edit or reject before anything is filed
- Run the visit loop: registration, vitals, consultation, prescription, pharmacy, follow-up

## Making things run better

- Find what slows the practice down — usually re-typing and re-reading
- Remove the re-typing; keep one record per patient on the NHS number
- Leave an audit trail: who read it, who coded it, who changed it



**Faster, not busier.**
The same people, doing less re-typing.



The human stays in charge of the clinical decision. The machine does the reading.

DIGITAL, THE RIGHT WAY

# Paper to structured data — without breaking the practice

## Going digital, the right way

- Easier for patients — nothing new to learn at the desk
- Simpler admin — one place a letter becomes a coded fact
- Better decisions — trends and recalls that actually populate

## Our focus clients

- GP practices with a heavy correspondence inbox
- Primary-care networks and PCNs sharing admin
- Community pharmacies dispensing against those prescriptions
- Small clinics still running on paper registers



Ingest → propose → confirm → file. Four steps, one audit trail.



Start narrow: one practice, one inbox, thirty days.

WHY US · AND WHAT WE MEASURE

# A pilot you can judge in 30 days

## Why work with us

- We understand clinical coding quality — it is the day job, not a side claim
- No one-size-fits-all template; the workflow is set by the practice
- Visible results: numbers a practice manager can read off one page
- We listen first, then build — and we say what we cannot do

## What we measure

- **Minutes per letter** — before versus after
- **Codes accepted** — share a clinician keeps unchanged
- **Recalls closed** — by day 30, not rolled into next month

If those three do not move, the pilot failed and we say so.

**UK deployment gates we do not hold yet:** DTAC · DSPT · DPIA under UK GDPR · Cyber Essentials or ISO 27001 · DCB0129 with a named Clinical Safety Officer.

# Let's build better care together.

One practice. One inbox.
Thirty days.

**Abhilashgonela886@gmail.com**

**+44 7344 796772**

**linkedin.com/in/abigonela**

### See the loop in 30 seconds

Six clickable screens, sample data, no sign-up.

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Independent project. Not affiliated with the NHS. No DTAC, DSPT or DCB0129 claim is made.